June 16, 2025

Dr. Mehmet Oz
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Dr. Thomas Keane
Department of Health and Human Services,
National Coordinator for Health Information Technology
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On behalf of J Michael Consulting (JMC) we are pleased to submit comments on the CMS and ASTP **Request for Information; Health Technology Ecosystem** (file code CMS-0042-NC).

JMC is a small business providing public health informatics consulting and support in laboratory informatics, surveillance informatics, and interoperability. We work closely with state, tribal, local and territorial public health agencies to improve the effectiveness and efficiency of these critical programs. We also work closely with federal partners and public health associations to provide broad support to the public health informatics and interoperability communities. Our goal is to bring science and technology together to benefit both individual patients and whole populations and are submitting comments with this goal in mind.

If you have any questions or wish to engage in further conversation, please reach out to our JMC Public Health Informatics Fellow Craig Newman.

Sincerely,

Jon M. Lipsky
Founder and Chief Executive Officer

General Comment

The recent Request for Information (RFI) issued by the Centers for Medicare & Medicaid Services (CMS) and the Office of the National Coordinator for Health Information Technology (ONC) underscores the need for a more interconnected and patient-centric health technology infrastructure. We would like to emphasize the critical importance of integrating public health perspectives into the ongoing discussions about the future of the health IT ecosystem. Public health entities, including state and local health departments, play a pivotal role in disease prevention and control and their involvement in health IT initiatives ensures that public health data is accurately captured, shared, and utilized to inform public health strategies and policies. We urge CMS and ASTP to actively engage public health stakeholders in the development and implementation of health IT policies and systems. By fostering collaboration between individuals, healthcare providers, payers, technology developers, and public health entities, we can create a more robust health IT ecosystem that serves the needs of all Americans.

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

We see CMS support for public health activities related to the growth and implementation of a digital tool ecosystem as being key to ensuring that the carefully curated data maintained by public health is available to Medicare beneficiaries and their caregivers. However, public health agencies need support to design, develop, test and implement technical solutions to meet the needs of Medicare served individuals. As well, CMS and other data sources including healthcare organizations and payers must be encouraged to meaningfully engage with public health to implement real-world exchange of data using standards-based approaches so that complete, longitudinal data is available to beneficiaries. It is crucial that trust is developed between CMS, healthcare providers, payers, and public health entities to ensure that appropriate requests from public health for authorized data sets are responded to in a complete and timely fashion by data holders. This can best be accomplished by clear support by CMS and other federal partners for data sharing with public health.

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

Historically, the public health experience has been that patient identification and matching is a complex but vital piece of public health data management and utilization. We strongly recommend including the public health perspective in deliberations regarding the implementation of digital identity within the healthcare realm. Too often demographics are either incomplete or inconsistent for definitive patient matching without costly human intervention. Integrating the needs of public health with those of other sectors of healthcare and

government will be critical to ensuring that jurisdictions and their trading partners implement digital identity functionality in a standards-based method.

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

The contents of USCDI meet the basic requirements of many public health program data needs, however, it is important to continue to raise the floor for data availability. By expanding electronic access to all data classes and elements used by public health, the burden and security risks of manual data access will be reduced for both public health and their data submitters. Having a clear pathway to transition data defined by USCDI+ (particularly the Public Health domain) to USCDI will create greater certainty regarding future data access capabilities. One clear area of focus that would greatly benefit public health data access is that of maternal and child health. Many public health programs rely heavily on access to pre-, peri- and post-natal data to provide support and services to mothers and their newborns. We encourage CMS and their federal partners to emphasize the need for this data through USCDI and other mechanisms which drive real-world exchange of data through standards-based approaches.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The 315(f) requirements supporting data exchange with public health have been instrumental in the development and implementation of standards-based data sharing. We encourage continued expansion of criteria to fully support all public health data needs. We also strongly support coordination between federal agencies to continue to expand incentives, such as Promoting Interoperability, for trading partners such as payers and healthcare organizations to implement real-world connections with public health.

TD-9. Regarding certification of health IT:

d. What policy changes could CMS make so providers are motivated to respond to API based data requests with best possible coverage and quality of data?

The development and implementation of FHIR subscription functionality could be transformational for many aspects of data sharing including public health reporting and case investigation. The Assistant Secretary for Technology Policy (ASTP) made FHIR Subscription a clear priority in the HTI-2 proposed rule. CMS and their federal partners can continue to support this avenue of exchange through carefully scoped regulations encouraging the development and implementation of these capabilities. Further, a clear vision for the inclusion of public health as a data subscriber should be shared with the larger healthcare community, signaling the importance of meaningfully engagement with public health.